

General workstation and hygiene standards

The station must be brought up to standard before the client arrives and returned to that same standard immediately after they leave, before it's used again. "Before and after" isn't just tidiness - it's the two checkpoints where contamination risk is highest: setup (unknown surfaces) and breakdown (blood and ink residue present).

Wash hands with soap and water, or an alcohol-based hand disinfectant if hands aren't visibly soiled, for at least 20–30 seconds before gloving. Gloves go on only after hands are fully dry. Gloves get changed, and hands rewashed or redisinfectant, any time they're contaminated - this includes touching your phone, adjusting equipment not part of the sterile setup, or moving between distinct steps of the procedure (e.g. shaving, then tattooing).

Personal protective equipment (gloves)

Single-use nitrile gloves, powder-free, certified to EN 455-1 to 455-4 (freedom from holes, physical properties, biocompatibility, and performance for single-use medical gloves) and EN ISO 374-5 (protection against bacteria and viruses, marked with the "virus" pictogram on the box). Nitrile is used rather than latex to avoid allergy risk to client or tattooist. Gloves are changed between every distinct step, contact, or contamination event as already described in the general hygiene section above (this section documents which gloves and which codes, not when to change them, since that's already covered).

If concentrated disinfectant is handled outside of gloved client contact (e.g. mixing or applying surface disinfectant), a chemical-resistant glove certified to EN ISO 374-1 Type B is used for that task instead of the examination glove.

Two-stage surface and instrument decontamination

1. SURFACE 1 (detergent) first, to mechanically remove dirt and dust - and, on a used station, organic matter like blood or ink. Disinfectants don't work reliably through visible soil, since organic material can shield pathogens from the active ingredient.
2. SURFACE 2 (alcohol disinfectant) second, applied to the now-clean surface, left for the manufacturer's stated contact time rather than wiped off immediately, since disinfectants need dwell time to actually kill pathogens.

Sterile barriers for reusable equipment. Anything reusable that will be touched during the session - machine grips, clip cords, spray bottles, trolley surfaces, arm rests - needs a single-use barrier: a sealed sterile bag or stretch film applied fresh for each client. This stops gloved hands, which will inevitably contact ink, blood, or skin, from directly touching a surface that has to survive to the next client. Anything that can't practically be barrier-wrapped should be single-use and disposed of after the session rather than reused unprotected.

Needle stick accidents

Take immediate action after a needle stick accident, following the procedure in Annex B of the norm. In practice this means: let the wound bleed briefly rather than squeezing it, rinse with water or water and soap, disinfect the wound, and cover it. Contact your GP, the GGD, or the hospital. Treatment of a possible infection should ideally start within 1 hour and no later than 72 hours after the accident. The incident is logged (date, circumstances, action taken) and first-aid materials for small wounds, such as quick dressings, wound plasters and a wound disinfectant, are kept visibly available at the studio at all times.



During the tattooing procedure

1. Wash and disinfect hands, then glove up before touching the client or any sterile equipment.
2. Apply tool 3 (foam) and shave with a disposable razor. The razor is single-use, disposed of immediately after - never reused, even on the same client for a later touch-up.
3. Wash the area with product 4 (Green Soap) to remove surface oils, sweat, and residual dirt before disinfection - the same clean-before-disinfect logic as the surface protocol.
4. Degrease and disinfect the skin with product 5, immediately before the needle contacts skin, to minimize bioburden at the puncture site.
5. Open cartridges and pour colors in front of the client. This isn't just procedure - it's visible proof to the client that sterile packaging was opened specifically for them and ink wasn't reused from a prior session. Any poured ink is single-use and discarded at the end, never returned to the bottle.
6. Use only studio-provided materials during the session - petroleum jelly (reduces friction, helps stencil/ink transfer), Green Soap (ongoing cleaning as the piece progresses), and Healing Second Skin (applied per the studio's aftercare protocol, typically once the piece is finished).

Cleaning the workstation after a tattoo session

1. Dispose of used cartridges and razors in a special sharps/biohazard container - never general waste.
2. Use clean and dirty (gloved) hands to remove disposables and barrier protection: one hand handles contaminated items, the other stays clean, and both gloves come off last, from the wrist.
3. Apply the healing film, following the administrator's instructions - ideally given to the client before they leave.
4. Use agent 1 to wash surfaces, then agent 2 to disinfect - same detergent-then-disinfectant sequence as the general standard, resetting the station to its starting condition.

Waste management

Sharps and biohazard waste (needles, cartridges, razors) go into the dedicated sharps/biohazard container as already described, never into general waste.

Used towels and other linens that have been in contact with blood or ink during a session are kept separate from both the sharps container and general waste at each individual workstation, in their own closed bag or bin at the station, from the moment they're used until they're removed for laundering or disposal. They are never combined with the sharps bin or with unrelated general waste, and never carried between stations before being bagged.

Guidelines for guest tattoo artists

The studio's permit covers this location, not any individual tattooist, so while you're working here you're covered under our permit, but you're personally responsible for following these guidelines in full, the same as our resident artists.

Before your first session, you will receive a short orientation covering where sterile supplies, disposable barriers, and disinfectants are stored, where the sharps/biohazard container is located, and where the first-aid materials for needle stick accidents are kept.

Anything reusable you bring with you (machines, grips, clip cords) must be brought up to the same standard as our own equipment before it touches a client: cleaned, disinfected, and barrier-wrapped, following the two-stage process described elsewhere in this handbook. If you bring sterile-packaged items from your own supplier, check the seal is intact before opening it in front of the client. Nothing from outside goes straight onto a client without this check.

